National Cancer Control Programme (NCCP) 00569 · clinician draft

Pembrolizumab + pemetrexed + cisplatin

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Maintenance: pembrolizumab + pemetrexed

Active medicines: pembrolizumab + pemetrexed

Emetogenic risk: low

Medicine	Dose / route	When
Folic acid	350–1000 micrograms orally daily	At least five doses in seven days before first pemetrexed; throughout treatment and 21 days after the final dose
Vitamin B12 (hydroxocobalamin)	1000 micrograms intramuscular (IM)	Week before first pemetrexed and every three cycles thereafter Later doses may be on the pemetrexed day.
Pemetrexed rash prophylaxis: dexamethasone equivalent	4 mg orally twice daily	Day before, day of and day after pemetrexed This is the regimen rash schedule; prescriber/pharmacy must reconcile it with antiemetic dexamethasone into one unambiguous prescription, including aprepitant interaction. Do not simply add both courses.
Low-risk antiemetic / rescue plan	Individual prescription	Around pemetrexed Account for the existing rash-prophylaxis steroid. Do not automatically continue induction aprepitant/olanzapine/high-risk steroid course.

Additional phase-specific support

- Continue daily folate, B12 every three cycles and pemetrexed rash prophylaxis.
- Cisplatin and its hydration protocol end after induction; reassess supportive prescriptions.
- Keep immune, renal, glucose and thyroid monitoring.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00569 v5 · pNCCP 00569 treatment and supportive-care sections; NCIS V8 · NCCP NCIS Medical Oncology antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required